SEMINAR PRESENTATION

Vomiting in Pregnancy: Nausea and Vomiting of Pregnancy in Ayurveda

Management of Garbhini Chardi and its classical formulations

Dr. Nidhi Sanjay Patel

Guide / Supervisor
Dr. Kola Vijaya Kumari, M.D. (Ayu), Professor & H.O.D.

Venue
Department of Prasuti Tantra & Stree Roga seminar series

Year
2023

> *Nearly three-quarters of pregnancies involve nausea. This seminar covers what Ayurveda offers — and what it deliberately withholds.*

Abstract

Nausea and vomiting of pregnancy is one of the earliest and most universal symptoms of gestation, described in the classical texts as Garbhini Chardi. This seminar presents the Ayurvedic understanding of vomiting in pregnancy, the doshic basis of the condition, the classical diagnostic and therapeutic approach, the role of Ahara-Vihara and the specific formulations indicated in pregnancy with their indications, precautions and rationales, and the differentiation from the pathological vomiting of later pregnancy such as hyperemesis gravidarum. The presentation also addresses the limits of classical management and the point at which modern intervention is required, aiming to give the postgraduate audience a safe and practical synthesis.

Nausea and vomiting of pregnancy · Garbhini Chardi · Prasuti Tantra · Ayurveda · Maternal care · Drug review · Safe in pregnancy

Cite this work

APA 7TH

Dr. Nidhi Sanjay Patel (2023). Vomiting in Pregnancy: Nausea and Vomiting of Pregnancy in Ayurveda [Seminar presentation]. Department of Prasuti Tantra & Stree Roga seminar series.

VANCOUVER

1. Dr. Nidhi Sanjay Patel. Vomiting in Pregnancy: Nausea and Vomiting of Pregnancy in Ayurveda [seminar presentation]; 2023; Department of Prasuti Tantra & Stree Roga seminar series.

Full text

Reproduced below is the complete text of the source document as submitted, reformatted for reading on the web and in print. The original file is also available alongside this PDF.

Topic of Presentation

VOMITING IN PREGNANCY

PRESENTED BY:

Dr. NIDHI PATEL MS 2nd year dept. of PSR

Guided by

DR. KOLA VIJAYA KUMARI

Professor & HOD

MD(AYU)

VOMITING IN PREGNANCY

Vomiting is a symptom which may be related to pregnancy or may be a manifestation of some medical-surgical-gynecological complications, which can occur at any time during pregnancy.

The causes of vomiting in pregnancy can be classified as follows :

EARLY PREGNANCY

LATE PREGNANCY

Related to pregnancy (vomiting of pregnancy)

Simple vomiting (morning sickness, emesis gravidarum)

Hyperemesis gravidarum (pernicious vomiting)

Associated with pregnancy

Related to pregnancy

Continuation or reappearance of simple vomiting of pregnancy

Acute fulminating preeclampsia

Associated with pregnancy

Medical-surgical-gynecological causes in early pregnancy

Hiatus hernia

ASSOCIATED WITH PREGNANCY

MEDICAL

SURGICAL

GYNECOLOGICAL

Intestinal infestation

Urinary tract infection

Hepatitis

Ketoacidosis of diabetes

Pyelonephritis, uremia

Appendicitis

Peptic ulcer

Intestinal obstruction

Cholecystitis

Pancreatitis

Twisted ovarian tumor

Red degeneration of fibroid

The vomiting is related to the pregnant state and depending upon the severity, it is classified as:

2198370160655

VOMITING IN PREGNANCY 0

VOMITING IN PREGNANCY 2940050627854003451464131387O

HYPEREMESIS GRAVIDARUM OR SEVERE TYPE 0

HYPEREMESIS GRAVIDARUM OR SEVERE TYPE 9298641329503

SIMPLE VOMITING OF PREGNANCY OR MILDER TYPE 0

SIMPLE VOMITING OF PREGNANCY OR MILDER TYPE 1660675973903001661061964378004177632964136

SIMPLE VOMITING :-

(Syn: morning sickness, Emesis gravidarum)

The patient complains of nausea and occasional sickness on rising in the morning.

Slight vomiting is so common in early pregnancy (about 50%) that it is considered as a symptom of pregnancy.

It may, however, occur at other times of the day.

The vomitus is small and clear or bile stained. It does not produce any impairment of health or restrict the normal activities of the women.

The feature disappears with or without treatment by 12–14th week of pregnancy.

High level of serum human chorionic gonadotropin, estrogen and altered immunological states are considered responsible for initiation of the manifestation, which is probably aggravated by the neurogenic factor.

MANAGEMENT:-

Assurance is important. Taking of dry toast or biscuit and avoidance of fatty and spicy foods are enough to relieve the symptoms in majority.

Supplementation with vitamin B1 100 mg daily is helpful.

If the simple measures fail, antiemetic drugs — trifluoperazine (Espazine) 1 mg twice daily is quite effective. Promethazine and ondansetron can be used.

Patient is advised to take plenty of fluids (2.5 L in 24 hours) and fruit juice.

HYPEREMESIS GRAVIDARUM

Definition: -

It is defined as vomiting in pregnancy, severe enough to produce dehydration, weight loss and ketosis from starvation.

It is a severe type of vomiting of pregnancy which has got deleterious effect on the health of mother and/or incapacitates her in day-to-day activities.

The adverse effects of severe vomiting are—dehydration, metabolic acidosis (from starvation) or alkalosis (from loss of hydrochloric acid), electrolyte imbalance (hypokalemia) and weight loss.

Epidemiology :-

Up to ninety percent of women experience nausea during pregnancy.

Studies showed that approximately 27 to 30 percent of women experience only nausea, while vomiting may be seen in 28 to 52 percent of all pregnancies.

The incidence of hyperemesis gravidarum ranges from 0.3 to 3 percent, depending on the literature source.

Etiology :-

The etiology is obscure but the following are the known facts:

It is mostly limited to the first trimester

It is more common in first pregnancy, with a tendency to recur again in subsequent pregnancies (15%)

Younger age

Low body mass

History of motion sickness or migraine

It has got a familial history — mother and sisters also suffer from the same manifestation

It is more prevalent in hydatidiform mole and multiple pregnancy

It is more common in unplanned pregnancies but much less amongst illegitimate ones.

Women with hyperemesis gravidarum, often suffer from transient form of hyperthyroidism (clinical or subclinical).

Theories: -

Hormonal:

Excess of chorionic gonadotropin or higher biological activity of hCG is associated. This is proved by the frequency of vomiting at the peak level of hCG and also the increased association with hydatidiform mole or multiple pregnancy when the hCG titer is very much raised

High serum level of estrogen. Estradiol levels increase early in pregnancy and decrease later, mirroring the typical course of nausea and vomiting in pregnancy. Additionally, nausea and vomiting are the known side effects of estrogen-containing medications. As the level of estrogen increases, so does the incidence of vomiting.

Progesterone excess leading to relaxation of the cardiac sphincter and simultaneous retention of gastric fluids due to impaired gastric motility. It is well-known that the lower esophageal sphincter relaxes during pregnancy due to the elevations in estrogen and progesterone. This leads to an increased incidence of gastroesophageal reflux disease (GERD) symptoms in pregnancy, and one symptom of GERD is nausea.

Other hormones involved are: thyroxine, prolactin, leptin and adrenocortical hormones.

Psychogenic :-

It probably aggravates the nausea once it begins. But neurogenic element sometimes plays a role, as evidenced by its subsidence after shifting the patient from the home surroundings.

Dietetic deficiency :-

Probably due to low carbohydrate reserve, as it happens after a night without food. Deficiency of vitamin B6 , vitamin B1 and proteins may be the effects rather than the cause.

Allergic or immunological basis

Whatever may be the cause of initiation of vomiting, it is probably aggravated by the neurogenic element. Unless it is not quickly rectified, features of dehydration and carbohydrate starvation supervene and a vicious cycle of vomiting appears.

right1120600

METABOLIC, BIOCHEMICAL AND CIRCULATORY CHANGES:

The changes are due to the combined effect of dehydration and starvation consequent upon vomiting.

Metabolic:

Inadequate intake of food results in glycogen depletion. For the energy supply, the fat reserve is broken down.

Due to low carbohydrate, there is incomplete oxidation of fat and accumulation of ketone bodies in the blood.

The acetone is ultimately excreted through the kidneys and in the breath. There is also increase in endogenous tissue protein metabolism resulting in excessive excretion of nonprotein nitrogen in the urine.

Water and electrolyte metabolism are seriously affected leading to biochemical and circulatory changes.

Biochemical:

Patients develop acidosis (due to starvation) and alkalosis from loss of hydrochloric acid and hyokalemia.

Loss of water and salts in the vomitus results in fall in plasma sodium, potassium and chlorides.

The urinary chloride may be well below the normal 5 g/L or may even be absent.

Hepatic dysfunction results in ketosis with rise in blood urea and uric acid.

Patient suffers from hypoglycemia, hypoproteinemia and hypovitaminosis.

Circulatory:

There is hemoconcentration leading to rise in hemoglobin percentage, RBC count and hematocrit values. There is slight increase in the white cell count with increase in eosinophils. There is concomitant reduction of extracellular fluid.

Sign & Symptoms:-

252649317454800From the management and prognostic point of view, the cases are grouped into:

12862312319990038919002317310012843882303400O

1497721076138

Vomiting occurs throughout the day. Normal day-to-day activities are curtailed. There is no evidence of dehydration or starvation. 4000020000

Vomiting occurs throughout the day. Normal day-to-day activities are curtailed. There is no evidence of dehydration or starvation. 391130670234300128659171232700347218029783 3

Late 00

Late 866565304184

Early 0

Early

2785110478155

Evidences of dehydration and starvation are present. 400000

Evidences of dehydration and starvation are present.

Symptoms:

Vomiting is increased in frequency with retching.

Urine quantity is diminished even to the stage of oliguria.

Epigastric pain, constipation may occur. Complications may appear if not treated.

Signs:

Features of dehydration and ketoacidosis: Dry coated tongue, sunken eyes, acetone smell in breath, tachycardia, hypotension, rise in temperature may be noted, jaundice is a late feature. Such late cases are rarely seen these days.

Vaginal examination and/or ultrasonography is done to confirm the diagnosis of pregnancy.

Investigations:

Urinalysis:

Quantity—small,

Dark color

High specifc gravity with acid reaction

Presence of acetone, occasional presence of protein and rarely bile pigments

Diminished or even absence of chloride.

Biochemical and circulatory changes:

Routine and periodic estimation of the serum electrolytes (sodium, potassium and chloride) is helpful in the management of the case.

Serum TSH, T3 and Free T4:

Women may sufer from transient phase of thyroid dysfunction (clinical or subclinical).

Ophthalmoscopic examination is required if the patient is seriously ill. Retinal hemorrhage and detachment of the retina are the most unfavorable signs.

ECG when there is abnormal serum potassium level.

Differential Diagnosis

The diagnosis of hyperemesis gravidarum is clinical and largely a diagnosis of exclusion. The list of potential differential diagnoses for patients with similar symptoms is quite extensive. It can include:

Gastrointestinal Conditions

Gastroenteritis

Gastroparesis

Achalasia

Biliary tract disease

Hepatitis

Intestinal obstruction

Peptic ulcer disease

Pancreatitis

Appendicitis

Genitourinary Conditions

Pyelonephritis

Uremia

Ovarian torsion

Kidney stones

Degenerating uterine leiomyoma

Metabolic Conditions

Diabetic ketoacidosis

Porphyria

Addison disease

Hyperthyroidism

Hyperparathyroidism

Neurologic Disorders

Pseudotumor cerebri

Vestibular lesions

Migraine headaches

Tumors of the central nervous system

Lymphocytic hypophysitis

Miscellaneous Conditions

Drug toxicity or intolerance

Psychologic Conditions

Pregnancy-related conditions

Acute fatty liver of pregnancy Preeclampsia

It is important to evaluate women presenting with (HG) for gestational trophoblastic disease (GTD) and multiple gestations as they may also present with severe nausea and vomiting in the first trimester of pregnancy.

The workup may begin with an obstetric ultrasound scan, which will confirm the diagnosis in most cases.

Other first-trimester obstetric concerns include ectopic pregnancy, which is more likely to include abdominal pain, syncope, or vaginal bleeding and can again be evaluated by obstetrical ultrasound and beta-hCG levels.

The onset of nausea and vomiting after nine weeks should spark concern for alternative diagnoses.

Preeclampsia, HELLP (hemolysis, elevated liver enzymes, and low platelets), and acute fatty liver of pregnancy typically present themselves during the late second or third trimester of pregnancy.

COMPLICATIONS:

Maternal:

The majority of the clinical manifestations are due to the effects of dehydration and starvation with resultant ketoacidosis.

Leaving aside those symptomatology, the following complications may occur which are fortunately rare nowadays.

Neurologic complications

Wernicke's encephalopathy, beriberi due to thiamine deficiency

Pontine myelinolysis

Peripheral neuritis

Korsakoff's psychosis.

Stress ulcer in stomach

Esophageal tear (Mallory-Weiss syndrome)

Jaundice, hepatic failure

Convulsions and coma

Hypoprothrombinemia due to vitamin K deficiency

Renal failure.

Effects on the fetus:

Fetus usually remains unaffected once the problem is resolved. Fetal risks may be due to low birth weight.

Management

The principles in the management are:

Maintenance of hydration

To control vomiting

To correct the fluids and electrolytes imbalance

To correct metabolic disturbances (acidosis or alkalosis)

To prevent the serious complications of severe vomiting

Care of pregnancy.

Treatment should be guided by the American College of Obstetrics and Gynecology (ACOG) Nausea and Vomiting in Pregnancy guidelines.

Initial treatment should begin with non-pharmacologic interventions such as switching the patient's prenatal vitamins to folic acid supplementation only, using ginger supplementation (250 mg orally 4 times daily) as needed and applying acupressure wristbands.

If the patient continues to experience significant symptoms, the first-line pharmacologic therapy should include a combination of vitamin B6 (pyridoxine) and doxylamine.

Three dosing regiments are endorsed by ACOG, including pyridoxine 10 to 25 mg orally with 12.5 mg of doxylamine three or four times per day, 10 mg of pyridoxine and 10 mg of doxylamine up to 4 times per day, or 20 mg of pyridoxine and 20 mg of doxylamine up to 2 times per day.

These are all FDA pregnancy category A medications.

Second-line medications include antihistamines and dopamine antagonists such as dimenhydrinate 25 to 50 mg every 4 to 6 hours orally, diphenhydramine 25 to 50 mg every 4 to 6 hours orally, prochlorperazine 25 mg every 12 hours rectally, or promethazine 12.5 to 25 mg every 4 to 6 hours orally or rectally.

If the patient continues to experience significant symptoms without exhibiting signs of dehydration, metoclopramide, ondansetron, or promethazine may be given orally.

Hospitalisation: -

If vomiting is not controlled, starvation results and the body's carbohydrate stores are used up for its immediate needs.

When the carbohydrate store is depleted, the fat stores are utilised(which gives rise to ketosis), symptoms of toxic manifestation follow.

At this juncture, the patients needs to be hospitalised.

Oral feeding is withheld for at least 24 hours after the cessation of vomiting.

During this period, fluid is given through intravenous drip method.

Fluid usage

In first 24 hours

• 1l Lactated Ringer's solution over 2h

• 1l Lactated Ringer's solution over 4h

• 1l 0.9% saline over 6h

• 1l 0.9% saline over 8h

Followed by

• 1l 0.9% saline every 8h as maintenance regimen

The total amount of fluid approximates 3 liters, of which half is 5% dextrose and half is Ringer's solution.

Extra amount of crystalloids equal to the amount of vomitus and urine in 24 hours, is to be added.

With this regime—dehydration, ketoacidosis, water and electrolyte imbalance are likely to be rectified.

Serum electrolyte should be estimated and corrected if there is any abnormality.

Enteral nutrition through nasogastric tube may also be given.

Hydrocortisone 100 mg IV in the drip is given in a case with hypotension or in intractable vomiting. Oral method prednisolone is also used in severe cases.

Ginger (Zingiber officinale)

Ginger root is reported to have chemoprotective activity in animal models. The methanol extract of ginger rhizome has been seen to inhibit the growth of 19 strains of H. pylori.

Ginger has shown superior efficacy compared to placebo without any adverse outcomes or side effects in cases of nausea and vomiting in pregnancy, but not for hyperemesis gravidarum.

Trials for hyperemesis gravidarum are lacking and only one trial has suggested a possible benefit.

Ginger-containing drugs are not approved by the United States Food and Drug Administration (US FDA) with concerns of potential effect on testosterone binding and thromboxane synthetase activity, but are remedies believed to improve symptoms and strongly recommended by the American College of Obstetrics and Gynecology (ACOG) 2004.

The dose to be given is 250mg of powdered ginger root administered orally every 6h.

Ginger is available in a number of forms such as tea, biscuits, confectionary, and crystals or sugared ginger, and although none has been subject to randomized controlled trials, there is some evidence that these forms of ginger may be beneficial and without adverse effects.

Fresh ginger can also be used as it is available at most grocery stores and health food stores.

To prepare it for eating, the brown skin is peeled off and ginger is washed under running water.

It is cut into small, thin slices to be eaten plain or added to salad, stew or soup. Women can put a slice of fresh ginger between their cheek and gum and chew on it throughout the day or they can nibble on small pieces of crystallized ginger.

Nursing care:

Sympathetic but firm handling of the patient is essential.

Social and psychological support should be extended.

left426466000Hyperemesis progress chart is helpful to assess the progress of patient while in hospital.

Daily record of pulse, temperature, blood pressure at least twice daily, intake-output, urine for acetone, protein, bile, blood biochemistry and ECG (when serum potassium is abnormal) are important.

Clinical features of improvement are evidenced by —

Subsidence of vomiting

Feeling of hunger

Better look

Normalization of blood biochemistry (electrolytes)

Disappearance of acetone from the breath and urine

Normal pulse and blood pressure

Normal urine output.

Diet:

Before the intravenous fluid is omitted, the foods are given orally.

At first, dry carbohydrate foods like biscuits, bread and toast are given.

Small but frequent feeds are recommended.

Gradually full diet is restored.

Termination of pregnancy is rarely indicated.

Intractable hyperemesis gravidarum in spite of therapy is rare these days.

In Ayurveda,

गर्भिणि छर्दि

निरुक्तिः-

छादयति मुखम् अर्दयति चाङ्गनीति छर्दिः ।

छद् अपवारणे अर्द हिंसायाम् अनयोः पृषोदरादित्वेन रूप सिद्धि। (विजयरक्षित)

That which fills up the mouth accompanied by compression and pain during vomiting is called chardi.

According to Chakrapani,

ऊर्ध्वातिदोषप्रवृत्तिरूपां छदिरूच्यते इति। (च.सं.चि. 20/1, चक्र)

Dosas when expelled in upward direction is called chardi.

According to Sushruta,

छादयन्नाननं वेगैरर्दरयन्नङ्गभञ्जनैः।

निरुच्यते छर्दिरिति दोषो वक्त्राद्विनिस्सरन्।। (सु.सं.उ. 49/6)

When vitiated dosas expel food along with dosas from āmāśaya via mouth, filling it up during the process of expulsion, it is called chardi.

निदान (Etiology)

नार्याधापन्नासत्त्वाया इति गर्भिण्याः, तस्या गर्भोत्पीडनेन वातवैगुण्याच्छर्दिः। (मा.नि. 15/3, मधु,व्या.)

Madhukoşa (commentator) says that the vitiated vayu (by foetus) is pushed upwards causing dauhrida chardi.

दौहदजा दौहदविमाननाज्जाता, अन्ये दौहजां गर्भनिमित्तामाहुः। (सु.सं.उ. 49/12, डल्ह.टी.)

Dalhaṇa says chardi is due to unfulfilled-desires in pregnancy or chardi which is due to garbha is dauhrda chardi.

नार्याश्चापन्नसत्त्वायाः....। बीभत्सजा दौहदजाऽऽमजा च....। (सु.सं.उ. 49/4)

Sushruta has mentioned pregnancy as one of the cause for chardi.

पूत्यमेध्याशुचि द्विष्टदर्शन श्रवणादिभिः।

तप्ते चित्ते हृदि क्लिष्टे छर्दिर्दिष्टार्थयोगजा।

वातादीनेव विमृशेत्कृमितृष्णाऽऽमदौहंद।। (अ.सं.नि. 5/38-39, अ.ह.नि. 5/36-37)

Sight, smell etc of putrifying, unpleasant, unhygienic, envying things disturbs the mind and heart produces vomiting.

In vomiting due to krmi, thirst, ama, dauhrda the symptoms corresponding to vāta etc dosas should be recognized.

छर्दि is one of the व्यक्त गर्भ लक्षण.

मुखप्रसेक, हल्लास is सद्योगृहीत गर्भ लक्षण.

Causes of hyperemesis can be grouped into three:

1. वातवैगुण्यं (neurological/psychological)

2. दौडदावमानन (nutritional deficiency-folic acid, pyridoxine etc.)

3. गर्भनिमित्त (maternal hormones/idiopathic)

Hence vomiting in pregnancy can be described as:

• मुखप्रसेक/हल्लास/छर्दि as a symptom can be termed as emesis gravidarum or morning sickness.

• दौडदजन्य छर्दि can be considered as hyperemesis gravidarum.

• दोषज उर्दि (Kashyapa) associated vomiting during pregnancy.

Samprapti of Garbhini chardi :-

Garbha vrudhi & Douhrauda Avamana

281139734480

Utklishta Dosha

281165729339

Vilomagati of utklishta doshas by vyana & udana

28145104890

Mukhapurana

281349532956

Garbhini Chardi

चिकित्सा सूत्र (Line of treatment)

दौहदजास्ता यथास्वं वाताद्यौषधेन च शमंयान्ति। (अ.सं.चि. 8/3, इन्दु टी.)

दौहदस्थां वमिं हयैः कांक्षितैर्वस्तु भिर्जयेत्। (यो.र. छर्दि.)

.....दौहदीं कांक्षितैः फलैः। (सु.सं.उ. 49/25)

इष्टदौहदजां फलैः। (भा.प्र.वि. 17/28)

अनुकूलोपचारेण याति द्विष्टार्थजा शमम्। (अ.ह. चि. 6/21)

Vomiting due to pregnancy should be treated by providing agreeable desired articles and other management should be done considering relative predominance of different dosha.

According to yogaratnakar,

शुण्ठिविल्वकषायं तु यवसक्नुसमन्वितम्।

गर्भिणीं पाययेद्वैद्य श्छर्यतीसारनाशनम्।

कुस्तुम्बरीणां कल्कं तु तण्डुलोदकसंयुतम्।।

पिवेत्सशर्करं हां गर्भिणीच्छर्दिनिवारणम्।

बिल्वमज्जा च लाजाम्बु पिवेच्छर्दिषु गर्भिणी।। (यो.र. स्त्रीरोग. 9,11-12)

Sunthi and vilva kashaya with parched barley flour cures diarrhoea and vomiting.

Dhanyaka paste with rice water and sugar.

Bilwamajja (pulp) with lajjambu (liquid from parched paddy).

According to Harita,

पीतोभूनिम्ब कल्कश्च शर्करासमभावितः। छर्दि हरेत्। (हा.सं.तृ. 51/6)

Use of bhunimba kalka and sugar (equal in quantity) cures vomiting.

According to Rasavagbhatta,

लाजैलाकोलमज्जाम्बु निपीतं वान्तिनाशनम्। (र.र.स. 20/88)

Kwatha of laja, ela, kola majja cures chardi in garbhinī.

दोषानुसार छर्दि चिकित्सा (Treatment for vomiting according to doṣha)

वातज छर्दि चिकित्सा (Treatment for vātaja chardi)

अथ छर्दि चिकित्सां तु प्रोच्यमानां निबोधत। मातुलुङ्गरसो लाजाः कोलमज्जा तथाऽञ्जनम्।।

तथा दाडिमसारश्च शर्करा क्षौद्रमेव च। एष वातात्मिकां छर्दि हन्ति लेहो विशेषतः ।।

दाडिमाम्लो रसः पक्को हृद्यो लवणवर्जितः। वातच्छर्दिहरो राजन् ! माहिषो वा सुसंस्कृतः।।

(का.सं.खि. 10/118-120)

Vataja chardi is treated with matulunga swarasa, parched paddy, kernel of kola, anjana, dadimasara, sugar and honey.

Dadima cooked with salt free oja or mahisha mamsa rasa cures vataja chardi.

पित्तज छर्दि चिकित्सा (Treatment for pittaja chardi)

शर्करामधुसंयुक्तं लाजचूर्णसमायुतम्। चातुर्जातककल्केन हृद्यं पुष्पैः सुवासितम् ।।

पित्तच्छर्दिप्रशमनं हितं तण्डुलधावने। हिता लाजमयी पेया सिताक्षौद्रेण संयुता।

जाङ्गलो वा रसः पथ्यः शर्करा मधुरीकृतः।। (का.सं.खि. 10/121-122)

Laja churna is mixed with sugar and honey, to this kalka chaturjataka is added, to make it palatable, flavour with flowers cures pittaja chardi.

Laja peya with sugar and honey is beneficial.

Jangala mamsa rasa with sugar and honey is congenial.

श्लेष्मज छर्दि चिकित्सा (Treatment for shleshmaja chardi)

आम्रजम्बु प्रवालानि सितानि सुश्रुतानि तु। क्षौद्रयुक्तानि पेयानि श्लेष्मच्छर्धा विशेषतः।

भोजनार्थे हितं यूषं मुद्गैदांडिमसारि तम्।। व्यपेतस्नेहलवणं हृद्यं छर्दिविनाशनम्।। (का.सं.खि. 10/123-124)

Shita kashaya of tender mango leaves, jambu with honey.

Mudga yusha medicated with dadima with added salt and heated butter relieves vomiting and increases appetite.

सन्निपातज छर्दि चिकित्सा (Treatment for sannipätaja chardi)

सन्निपात समुत्थायां संसृष्टान्यवचारयेत्। (का.सं.खि. 10/125)

Combined treatment of the three doshas are adopted.

कृमिज छर्दि चिकित्सा (Treatment for krmija chardi)

क्रिमिजायां तु कर्त्तव्यं यत् पुरस्तात् प्रवक्ष्यते। वर्षाभूमूलनिष्काथं योजयेद्भद्रदारुणा।। (का.सं.खि. 10/126)

Treatment should be according to the predominance of specific dosha.

Kwatha of punarnavamula and bhadradaru with honey.

गर्भावस्थायाम् हल्लास चिकित्सा (Treatment for nausea in pregnancy)

पीतो भूनिम्बकल्कश्च...। हरेच्च हत्क्लेदं मधुना वा समन्वितः।। (हा.सं.तृ. 51/6)

Bhunimba kalka with honey cures nausea in pregnancy.

Conclusion: -

Health of a pregnant woman is of at most importance. A series of physiological and psychological changes happen in different stages of women, some of them may end in discomfort and trouble. So one should take care to treat this condition in initial stage and prevent complications. In our classics various simple, safe formulations have been explained which can easily be incorporated in day today life by a pregnant woman. They are easy to prepare, easily available.

REFRENCES:-

DC Dutta's textbook of Obstetrics

Mudaliar and Menon's Clinical Obstetrics

Textbook of Obstetrics by J B Sharma

Williams Obstetrics

Kashyap Samhita by P.V Tewari

Harita Samhita by Gyanendra Pandey

Prasuti Tantra obstetrics by Dr. Hemalatha Kapoorchand

Textbook of Prasuti tantra by Premavati Tiwari

Pubmed

Open-access copy published by the author at https://www.drnidhi.net/writing/vomiting-in-pregnancy.html
Dr. Nidhi Sanjay Patel — n71097p@gmail.com · Licensed CC BY 4.0